

CRITICAL ALERT: INPATIENT EMERGENCY ADVANCED CLINICAL LAB & BIOMARKER REPORT

|               |                     |                 |                          |
|---------------|---------------------|-----------------|--------------------------|
| Patient Name: | Mrs. Sunita Rao     | UHID / Reg No:  | AP-993012-2026           |
| Age / Gender: | 62 Yrs / Female     | Collected Date: | May 17, 2026 10:15 AM    |
| Referred By:  | Dr. Meera Reddy, DM | Ward Location:  | ICU - Bed 04 (Emergency) |

CLINICAL HISTORY & PRESENTATION

Patient was brought to the emergency department via cardiac ambulance presenting with acute substernal crushing chest pain radiating to the left arm, profound dyspnea, diaphoresis, and sudden oliguria. Known history of type-2 diabetes mellitus and chronic vascular hypertension. Clinical evaluation warrants immediate biochemical investigation due to high suspicion of acute decompensated coronary syndrome and concomitant critical organ failure.

CORE BIOCHEMISTRY & DIAGNOSTIC PANEL RESULTS

| Test Parameter            | Observed Value | Reference Interval | Units | Flag Status   |
|---------------------------|----------------|--------------------|-------|---------------|
| Cardiac Troponin I (cTnI) | 3.80           | 0.00 - 0.04        | ng/mL | CRITICAL HIGH |
| NT-proBNP                 | 4200           | < 125              | pg/mL | CRITICAL HIGH |
| Serum Creatinine          | 4.10           | 0.60 - 1.20        | mg/dL | CRITICAL HIGH |
| Blood Urea Nitrogen (BUN) | 68.0           | 7.0 - 20.0         | mg/dL | HIGH          |
| Fasting Plasma Glucose    | 285            | 70 - 100           | mg/dL | CRITICAL HIGH |
| Total WBC Count           | 16400          | 4000 - 11000       | /μL   | ELEVATED      |
| Hemoglobin                | 8.2            | 12.0 - 15.5        | g/dL  | CRITICAL LOW  |

## CRITICAL CARE CLINICAL INTERPRETATIONS

- **Cardiovascular Panel:** Severe escalation of Cardiac Troponin I (3.80 ng/mL) accompanied by profoundly high NT-proBNP confirms acute myocardial injury consistent with an ongoing acute myocardial infarction (Heart Attack) and secondary acute congestive heart failure.
- **Renal Panel:** Serum Creatinine at 4.10 mg/dL with a calculated eGFR of 14 mL/min indicates an abrupt development of Stage 5 Acute-on-Chronic Kidney Injury (Renal Failure) requiring immediate nephrology intervention and urgent dialysis evaluation.
- **Metabolic State:** Uncontrolled hyper-glycemia (285 mg/dL) places the patient at immediate risk of diabetic ketoacidosis. Critical microcytic hypochromic anemia (Hemoglobin 8.2 g/dL) complicates tissue oxygenation demands.

## PRIMARY CLINICAL DIRECTIVES

**THIS IS A HIGH-RISK, LIFE-THREATENING CRITICAL EMERGENCY. Immediate medical attention is required. Transfer patient to Cardiac ICU post-haste for continuous hemodynamic monitoring, emergent coronary angiography evaluation, anti-thrombotic therapy protocol, and emergent nephrology consult.**

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**Dr. V. Kapoor, MD, DM**

Chief Consultant Neuro-Cardiology Care  
Apollo Comprehensive Emergency Network

## CLINICAL REPORT: DIGITAL RADIOGRAPHIC CHEST X-RAY FINDINGS (PA VIEW)

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|---------------|----------------------|----------------|------------------------|
| Patient Name: | Mr. Rajesh Kumar     | UHID / Reg No: | FH-2026-884910         |
| Age / Gender: | 54 Yrs / Male        | Examined Date: | May 17, 2026 02:30 PM  |
| Referred By:  | Dr. A. K. Sharma, MD | Department:    | Pulmonary Medicine OPD |

### CLINICAL HISTORY & INDICATIONS

Patient presents with a persistent productive cough lasting for three weeks, continuous low-to-medium-grade fever fluctuating between 100.5°F and 101.8°F, localized right-sided pleuritic chest pain during deep inhalation, and progressive exertional fatigue. Patient reports a history of chronic tobacco smoking spanning 25 years.

### RADIOGRAPHIC EXAMINATION FINDINGS

- Lung Parenchyma:** A prominent, dense, irregular radio-opacity/consolidation measuring approximately 4.2 cm is clearly visualized in the right upper lobe zone. There are additional patchy non-homogeneous infiltrates present bilaterally in the bases of both lungs, highly indicative of local air-space disease.
- Pleural Spaces:** Blunting of the right costophrenic angle is noted, consistent with scant localized pleural effusion on the right side. Left pleural space appears unremarkable.
- Mediastinum & Hilar Architecture:** Visible widening of the superior mediastinum with structural prominence and enlargement of the right hilar lymph nodes. The cardiac silhouette shows mild enlargement (cardiomegaly) with a left ventricular dominance pattern.
- Trachea:** The thoracic trachea is clear but exhibits a very mild leftward shift, likely secondary to a minor structural mass-effect from the right upper lobe consolidation.

### RADIOLOGICAL IMPRESSION / INTERPRETATION

- \*\*Large consolidation/mass\*\*** in the right upper lobe. Differential diagnoses include primary active bacterial lobar pneumonia with necrotizing changes, or a primary endobronchial neoplastic mass lesion given the extensive history of tobacco consumption.
- \*\*Right-sided scant pleural effusion\*\*** with mild bilateral pulmonary venous congestion.
- \*\*Right hilar lymphadenopathy\*\*** and accompanying cardiomegaly.

### CLINICAL RECOMMENDATION MATRIX

**CLASSIFICATION: MEDIUM TO HIGH RISK STAGE.** This condition is progressive and warrants prompt diagnostic escalation. It is highly recommended to perform a Contrast-Enhanced Computed Tomography (CECT) scan of the chest and abdomen immediately to further differentiate between neoplastic mass and infectious consolidation. Expeditious pulmonology consultation is advised.

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**Dr. S. Mukhopadhyay, MD**  
Senior Consultant Radiologist  
Department of Diagnostic Imaging, Fortis

# APOLLO CLINICS & HEALTH CHECK

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Preventive Healthcare Division

## HEALTH STATUS: EXECUTIVES ANNUAL COMPREHENSIVE HEALTH CHECK PROFILE

|               |                          |                 |                       |
|---------------|--------------------------|-----------------|-----------------------|
| Patient Name: | Mr. Abhinav Mishra       | UHID / Reg No:  | AP-CLN-102948         |
| Age / Gender: | 34 Yrs / Male            | Collected Date: | May 17, 2026 08:00 AM |
| Referred By:  | Self / Corporate Package | Report Status:  | Final Verified        |

### CLINICAL INDICATION

Subject presented for a routine, voluntary annual preventative corporate health check screening. Asymptomatic individual with no current somatic complaints, no documented history of chronic diseases, and regular metabolic parameters.

### BIOMEDICAL LABORATORY EVALUATION

| Test Component Name         | Observed Value | Biological Reference Range | Units | Status Flag |
|-----------------------------|----------------|----------------------------|-------|-------------|
| Hemoglobin (Hb)             | 14.8           | 13.5 - 17.5                | g/dL  | NORMAL      |
| Total Leukocyte Count (WBC) | 6500           | 4000 - 11000               | /μL   | NORMAL      |
| Platelet Count              | 260000         | 150000 - 450000            | /μL   | NORMAL      |
| Fasting Blood Glucose       | 88             | 70 - 100                   | mg/dL | NORMAL      |
| HbA1c (Glycated Hemoglobin) | 5.4            | 4.0 - 5.6                  | %     | NORMAL      |
| Serum Creatinine            | 0.92           | 0.60 - 1.20                | mg/dL | NORMAL      |
| Blood Urea                  | 24.0           | 15.0 - 40.0                | mg/dL | NORMAL      |
| Total Cholesterol           | 182            | < 200                      | mg/dL | NORMAL      |

### PREVENTATIVE CLINICAL SUMMARY

All evaluated indices including hematology parameters, blood glucose regulation metrics, core renal parameters, and lipid profiles present safely within normal biological reference parameters. No critical abnormalities or active metabolic risk indicators are identified across the screened panels.

## CLINICAL RECOMMENDATION STATUS

**CLASSIFICATION: LOW RISK / HEALTHY PROFILE.** No immediate therapeutic medical attention is required. Patient is advised to maintain current healthy dietary habits, routine physical exercises, and repeat a standard screening evaluation in 12 calendar months.

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**Dr. Ramesh G., MD**

Consultant Physician

Apollo Preventive Health Check Division